INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Brigatinib Therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00562
Version 4

Continuous oral treatment with a seven-day lead-in

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Your record and contact card	16
05	Sources and further information	19

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Brigatinib is a targeted medicine intended to control ALK-positive advanced lung cancer. The choice depends on the cancer's ALK result, previous treatment, your health and your priorities.

ORAL ALK-TARGETED THERAPY

Brigatinib

Blocks an abnormal ALK growth signal

ALK alteration drives growth → Brigatinib blocks ALK signalling → Cancer growth may slow

An altered ALK gene can drive cancer-cell growth. Brigatinib blocks the activity of this signal. Testing confirms whether this target is present; the medicine can also affect healthy tissues, so regular checks and early symptom reporting matter.

Report breathing changes immediately, especially in the first week. Follow the exact tablet strength and lead-in plan; check all new medicines with your pharmacist.

Why brigatinib is being considered

Confirm the cancer test and previous treatment before choosing this medicine.

Two distinct treatment settings

Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

NCCP 00562 v4 · official regimen

What the medicine does

EMA Alunbrig product information and patient leaflet, current retrieved snapshot

How long treatment lasts

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

NCCP 00562 v4 · official regimen

Why this ALK-targeted medicine may be chosen

Clinical studies support brigatinib in selected ALK-positive advanced lung cancers before any ALK inhibitor and after crizotinib. The studies involved different treatment settings.

Trial averages cannot predict your outcome or establish which modern ALK inhibitor is best for you. Discuss your treatment choices and priorities.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact your oncology team immediately for new or worsening breathlessness, cough or fever, especially during the first week but also later. Do not wait for a booked check. Urgent advice is also needed for fainting, a very slow heartbeat, chest pain, severe headache or vision changes, marked muscle pain/weakness, severe abdominal pain, persistent vomiting, unusual thirst or frequent urination, jaundice or severe diarrhoea. Major breathing difficulty, collapse or severe chest pain needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show sequence, not equal durations. Continuous tablets do not create a chemotherapy cycle. The team confirms booking, review and any extra tests.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00562 v4 · official regimen · EMA Alunbrig product information and patient leaflet, current retrieved snapshot

THE WHOLE TREATMENT JOURNEY

Brigatinib Therapy: treatment and checks

Your treatment pattern

Continuous oral treatment with a seven-day lead-in

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ALK-positive advanced non-small-cell lung cancer (NSCLC) · before an ALK inhibitor or after crizotinib

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First seven days	Days 8–15 and first three months	Continuing treatment	Pause / stop / change
Tablets and lead-in NCCP 00562	Confirm indication and written dose	Usual 90 mg daily lead-in	Increase only if authorised; usual ongoing dose 180 mg daily	Daily treatment while beneficial and tolerated	Reason-specific restart; do not self-escalate
Breathing NCCP 00562 · SMPC	Pulmonary function and previous lung disease review	Report new cough or breathlessness immediately	Planned clinical checks on days 8 and 15	Monthly clinical assessment or sooner if symptoms	Follow recovery; early pneumonitis can prohibit escalation
Liver tests NCCP 00562	Baseline liver profile	Early extra tests if clinically needed	Every two weeks for first three months	As clinically indicated afterwards	Review abnormal tests before any restart

Care / check	Before starting	First seven days	Days 8–15 and first three months	Continuing treatment	Pause / stop / change
Heart and blood pressure NCCP 00562	Baseline electrocardiogram (ECG) and blood pressure	Symptoms require earlier assessment	Every two weeks during initial treatment period	Monthly or as clinically indicated	Review heart-rate/blood pressure (BP) medicines after treatment changes
Blood, kidneys and metabolic tests NCCP 00562	full blood count (FBC), renal profile, fasting glucose, creatine phosphokinase (CPK), amylase/ lipase	Extra tests if symptoms occur	Monthly planned tests	Continue monthly or sooner if indicated	Resolve abnormalities and review dose limitations
Benefit, medicines and priorities NCCP 00562 · SMPC	ALK test, previous treatment, interactions and consent	Confirm tablet supply and contacts	Check adherence, interactions, symptoms and daily function	Disease assessment and scans per Consultant plan	Agree next treatment, follow-up and ongoing precautions

What each check is for

1 Check the exact indication

Before the first dose

Treatment line and ALK result define this pathway.

Ask / act: Confirm written lead-in, monitoring and contact plan.

2 Early pulmonary surveillance

First week and scheduled day 8 / day 15 review

Early pulmonary events can occur before the first follow-up.

Ask / act: Report immediately; review whether escalation is still appropriate.

3 Ongoing tests

Every two weeks initially for selected tests; monthly for others

Liver, cardiac and metabolic monitoring have different cadences.

Ask / act: Confirm due window, booking team and result reviewer.

4

Restart or stop review

Any pause, dose change or final dose

Reason and duration of a break change the restart plan.

Ask / **act:** Obtain a new individual written plan; retain contraception and five-day sun precautions after stopping.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Starting, continuing and reviewing brigatinib

No dates or personal information are required. Your treating team confirms individual visits and changes.

Treatment or check	Timing in this view
Prepare	**Before starting** ALK, prior treatment, tests, medicines and consent
Seven-day lead-in	**First seven days** Usual 90 mg daily; watch breathing closely
Continuing tablets	**Days 8–15 and first three months → Continuing treatment** Usual 180 mg daily only if escalation is authorised; continue while helpful and tolerated
Tests and symptoms	**First seven days → Days 8–15 and first three months → Continuing treatment → Pause / stop / change** Scheduled monitoring and immediate reporting of new symptoms
Hold / restart / stop	**Pause / stop / change** Reason-specific medical review; do not restart or increase yourself

Do not restart or increase brigatinib yourself. A break of 14 days or more for a reason other than a side effect normally needs a new seven-day lead-in before the previously tolerated dose. A break because of a side effect follows a different recovery plan. Early lung inflammation can prevent the usual increase to 180 mg.

Columns show sequence, not equal lengths of time.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Taking treatment at home

Your prescription is the individual plan.



Taking the tablets

Swallow the tablets whole with water, with or without food. Do not crush, chew or split them. Avoid grapefruit and grapefruit juice. If a dose is missed or you vomit after it, do not take an extra dose; take the next dose at its usual time. Contact the team for ongoing vomiting or difficulty keeping fluids or medicine down.

Lead-in and dose increase

NCCP 00562 v4 · official regimen

After a pause

Do not restart or increase brigatinib yourself. A break of 14 days or more for a reason other than a side effect normally needs a new seven-day lead-in before the previously tolerated dose. A break because of a side effect follows a different recovery plan. Early lung inflammation can prevent the usual increase to 180 mg.

NCCP 00562 v4 · official regimen

Missed or vomited dose

Do not replace the dose or double the next dose. Resume at the next scheduled time and contact the team if symptoms prevent treatment.

NCCP 00562 v4 · official regimen

Checks before and during treatment

Some important reactions appear before the next appointment.

Baseline and ongoing checks

Before starting: ALK result, blood count, kidney/liver tests, breathing-function assessment, ECG, blood pressure, fasting glucose, muscle enzyme CPK and pancreatic enzymes. Review breathing symptoms on days 8 and 15, then monthly or sooner if needed. ECG and blood pressure are checked every two weeks initially, then monthly. Liver tests are every two weeks for the first three months, then as advised. Blood count, kidneys, fasting glucose, CPK and pancreatic enzymes are usually checked monthly. Scans follow your Consultant's plan.

NCCP 00562 v4 · official regimen

Breathing needs particular attention

Most early pulmonary reactions happen in the first seven days, but lung inflammation can occur later. Report new cough, breathlessness or fever straight away; the team may pause treatment while checking the cause.

Muscles, pancreas and glucose

Blood tests can identify changes before symptoms. Report muscle pain or weakness, severe abdominal pain, unusual thirst or frequent urination.

Other medicines, skin and family planning

Ask before changing other medicines or the brigatinib dose.

Interaction review

Some antibiotics, antifungals, antivirals, seizure medicines and herbal products can alter treatment. Your oncology pharmacist must review the complete medicine list. Do not change existing medicines yourself.

Sun protection

Avoid prolonged sun exposure during treatment and for at least five days afterwards. Use protective clothing, a hat, broad-spectrum sun protection factor (SPF) 30 or higher sunscreen and lip balm. Report severe rash or sun reaction.

Pregnancy, breastfeeding and fertility

Discuss fertility before treatment. Effective non-hormonal contraception is advised for women during treatment and for at least four months after the last dose; men with partners who could become pregnant need effective contraception during treatment and for at least three months afterwards. Tell the team promptly about possible pregnancy. Breastfeeding must stop during treatment.

Driving and daily tasks

Blurred vision, dizziness or fatigue can affect driving and machinery. Avoid these activities when affected and discuss persistent symptoms.

When to seek help

Follow your treating hospital’s urgent-contact instructions.

Call promptly

Contact your oncology team immediately for new or worsening breathlessness, cough or fever, especially during the first week but also later. Do not wait for a booked check. Urgent advice is also needed for fainting, a very slow heartbeat, chest pain, severe headache or vision changes, marked muscle pain/weakness, severe abdominal pain, persistent vomiting, unusual thirst or frequent urination, jaundice or severe diarrhoea. Major breathing difficulty, collapse or severe chest pain needs emergency help.

When you call

Give the medicine, current dose, when symptoms started and when the last dose was taken. Say whether you can drink, pass urine and manage daily activities; do not delay calling to collect details.

HSE cancer treatment side effects and urgent advice

Practical support

One written tablet plan

Keep the current dose, tablet strengths and next review clear. Ask the pharmacist about safe storage, handling and returning unused tablets. Do not share medicines.

Anti-sickness medicines only if needed

Brigatinib has minimal-to-low oral sickness risk. The oral NCCP guidance uses anti-sickness medicines as required; your team chooses a suitable medicine, dose and duration. Persistent vomiting still needs advice.

Food and other medicines

Take with or without food; avoid grapefruit. Check every prescription, herbal or non-prescription product, including St John’s wort, before starting or stopping it.

Skin and sunshine

Symptoms and practical support

Keep the 24-hour oncology contact available. Ask early for help with symptoms, eating, tablet supply, work, travel or emotional wellbeing.

Taking tablets / attending treatment

Practical treatment details

Prescriptions and supply

Check tablet strengths and the next supply before running out.

Questions to ask

Why does the dose start lower?

The initial seven-day lead-in is part of the brigatinib schedule and helps reduce early pulmonary reactions. Only increase when the team confirms it is appropriate.

What if I miss or vomit a dose?

Do not take an extra dose. Take the next scheduled dose at the usual time. Contact the team for ongoing vomiting or inability to keep fluids or medicine down.

Does the first-week breathing warning stop after day seven?

No. Early reactions are especially important, but lung inflammation can also happen later. Report new or worsening breathing symptoms immediately.

How long will I take brigatinib?

There is no fixed number of cycles. Your team reviews benefit, side effects, scans and your priorities and agrees whether to continue, change or stop.

Can I use this page to decide whether tests are safe for treatment?

No. The clinical team combines the protocol, symptoms, previous dose changes and organ function before prescribing.

What belongs in the consent discussion?

Your treatment aim, expected benefit and uncertainty, other options including not proceeding, serious and common harms, lasting effects, fertility and practical concerns. You can ask for more time or explanation and reconsider the decision.

Explain the plan back: Can you explain the seven-day lead-in, who authorises a dose increase or restart, what to do after a missed or vomited dose, which checks are due and how to get urgent help?

Brigatinib monotherapy · ALK-positive advanced non-small-cell lung cancer (NSCLC) Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Brigatinib alone · lead-in and continuing tablets

Active medicines: brigatinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Minimal-to-low oral emetogenic risk: antiemetic treatment as required only, with an individual prescribed choice/dose/duration and interaction review.
- No routine premedication is required by NCCP. NCIS V8 parenteral schedules should not be copied as routine daily prophylaxis for this oral medicine.
- Swallow the tablets whole with water, with or without food. Do not crush, chew or split them. Avoid grapefruit and grapefruit juice. If a dose is missed or you vomit after it, do not take an extra dose; take the next dose at its usual time. Contact the team for ongoing vomiting or difficulty keeping fluids or medicine down.
- Do not restart or increase brigatinib yourself. A break of 14 days or more for a reason other than a side effect normally needs a new seven-day lead-in before the previously tolerated dose. A break because of a side effect follows a different recovery plan. Early lung inflammation can prevent the usual increase to 180 mg.
- Discuss fertility before treatment. Effective non-hormonal contraception is advised for women during treatment and for at least four months after the last dose; men with partners who could become pregnant need effective contraception during treatment and for at least three months afterwards. Tell the team promptly about possible pregnancy. Breastfeeding must stop during treatment.
- Avoid prolonged sun exposure during treatment and for at least five days afterwards. Use protective clothing, a hat, broad-spectrum sun protection factor (SPF) 30 or higher sunscreen and lip balm. Report severe rash or sun reaction.
- Review creatine phosphokinase (CPK)/muscle, pancreatic, glucose, blood pressure/heart rate and liver results; separate medicine-specific management from prophylaxis.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Brigatinib monotherapy · ALK-positive advanced non-small-cell lung cancer (NSCLC) Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

NATIONAL CANCER CONTROL PROGRAMME (NCCP) 00562 · VERSION 4

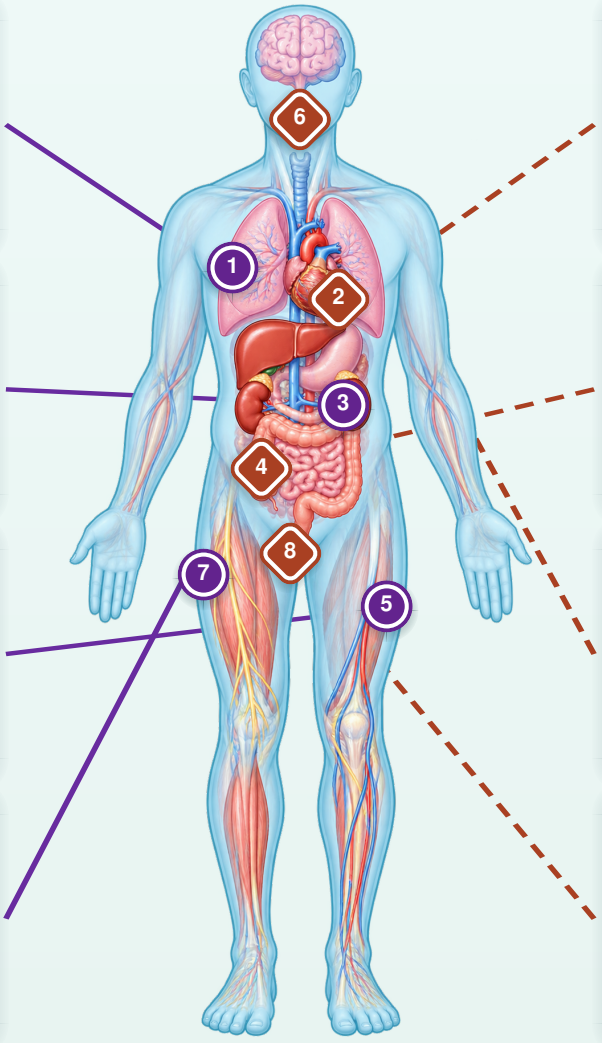
Brigatinib Therapy

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Oral ALK-targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Breathing

Brigatinib

New cough, breathlessness or fever needs immediate advice, especially during the first week.

2 Heart / blood pressure

Brigatinib

Report fainting, slow heartbeat, chest pain or severe headache.

3 Liver tests

Brigatinib

Blood tests check the liver; report jaundice, dark urine or marked illness.

4 Glucose / pancreatic enzymes

Brigatinib

Report unusual thirst, frequent urination or severe abdominal pain.

5 Muscles

Brigatinib

Report unexplained muscle pain or weakness.

6 Vision

Brigatinib

Report new or worsening blurred vision or other changes.

7 Skin / sun sensitivity

Brigatinib

Use sun protection and report severe rash or sun reaction.

8 Bowel / nausea

Brigatinib

Report persistent vomiting or severe diarrhoea; keep fluids up as advised.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team immediately for new or worsening breathlessness, cough or fever, especially during the first week but also later. Do not wait for a booked check. Urgent advice is also needed for fainting, a very slow heartbeat, chest pain, severe headache or vision changes, marked muscle pain/weakness, severe abdominal pain, persistent vomiting, unusual thirst or frequent urination, jaundice or severe diarrhoea. Major breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Breathlessness / cough / fever	☐	☐	☐	
Dizziness / fainting / slow heartbeat	☐	☐	☐	
Headache / blood-pressure symptoms	☐	☐	☐	
Muscle pain or weakness	☐	☐	☐	
Vision changes	☐	☐	☐	
Thirst / frequent urination	☐	☐	☐	
Abdominal pain / nausea / vomiting	☐	☐	☐	
Diarrhoea	☐	☐	☐	
Skin rash / sun sensitivity	☐	☐	☐	
Fatigue / appetite change	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact your oncology team immediately for new or worsening breathlessness, cough or fever, especially during the first week but also later. Do not wait for a booked check. Urgent advice is also needed for fainting, a very slow heartbeat, chest pain, severe headache or vision changes, marked muscle pain/weakness, severe abdominal pain, persistent vomiting, unusual thirst or frequent urination, jaundice or severe diarrhoea. Major breathing difficulty, collapse or severe chest pain needs emergency help. Emergency symptoms: 112 / 999.

Carry your treatment details

Brigatinib monotherapy · ALK-positive advanced NSCLC Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Brigatinib Therapy

NCCP 00562 v4

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact your oncology team immediately for new or worsening breathlessness, cough or fever, especially during the first week but also later. Do not wait for a booked check. Urgent advice is also needed for fainting, a very slow heartbeat, chest pain, severe headache or vision changes, marked muscle pain/weakness, severe abdominal pain, persistent vomiting, unusual thirst or frequent urination, jaundice or severe diarrhoea. Major breathing difficulty, collapse or severe chest pain needs emergency help.

https://sactcheck.com/patient/00562/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00562 v4 · p1–7; oral guidance Table 5 page 8 and Table 8 page 13**
   https://healthservice.hse.ie/documents/6895/562_Brigatinib.pdf
2. **EMA Alunbrig product information and patient leaflet, current retrieved snapshot**
   https://www.ema.europa.eu/en/documents/product-information/alunbrig-epar-product-information_en.pdf
3. **NCCP oral systemic anticancer therapy (SACT) antiemetic guidance V6, minimal-to-low risk (as required); NCIS V8 reviewed for scope**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS Medical Oncology antiemetics V8 (2025), parenteral scope**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **ALTA-1L final analysis: brigatinib versus crizotinib, 2021**
   https://pubmed.ncbi.nlm.nih.gov/34537440/
6. **ALTA final analysis: crizotinib-refractory ALK-positive non-small-cell lung cancer (NSCLC), 2022**
   https://pubmed.ncbi.nlm.nih.gov/36065449/
7. **HSE cancer treatment side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/